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# Assessing Australia's Health System

*A Health Systems Performance Assessment*
18th February 2025

***Health Systems Development Assessment*** *– Emmanuel, Siyuan, Taras, Ria, Jiawei, & Shelly*

# Background

## Origins & Development

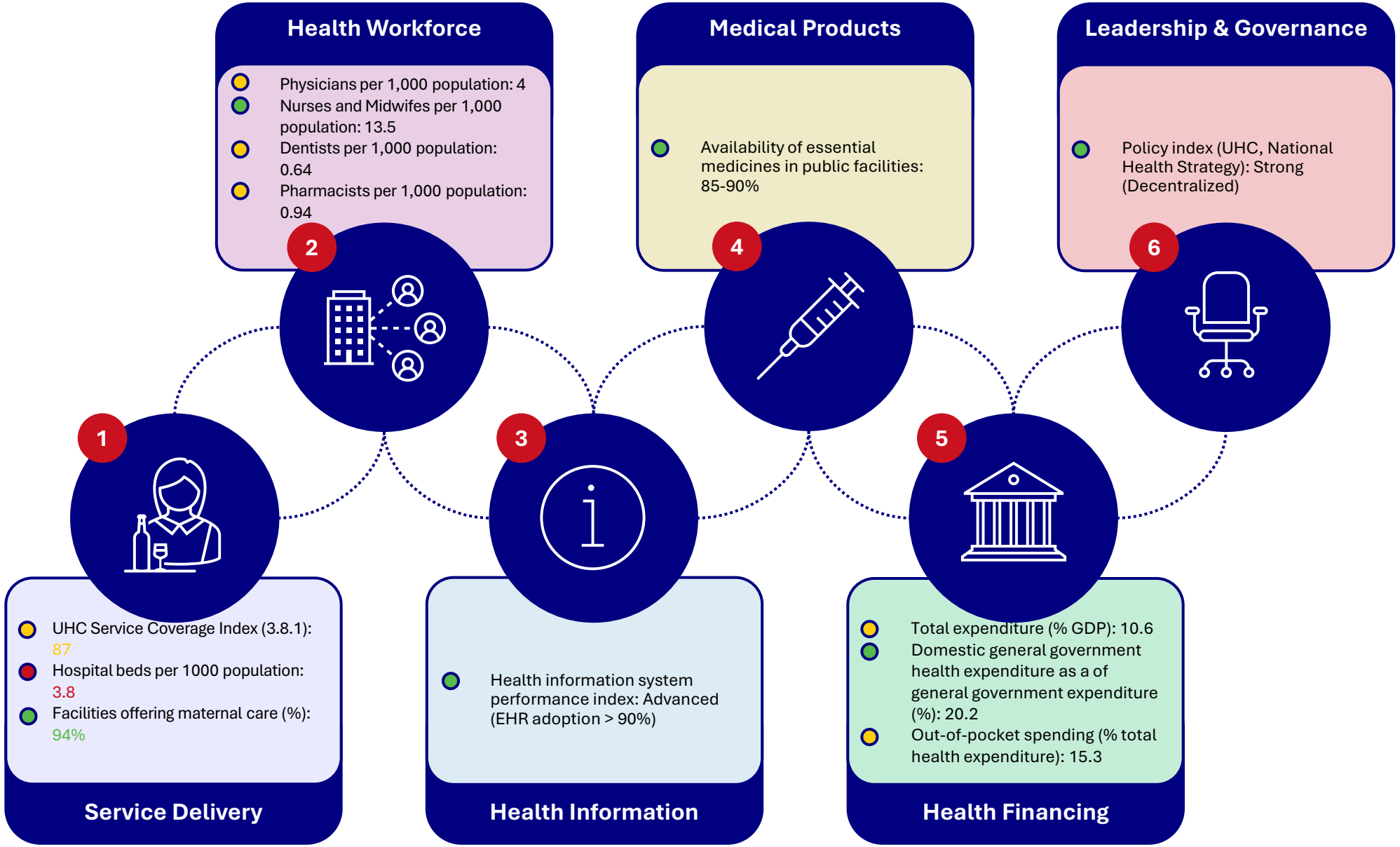
- **1973** — Universal care is first introduced, though rejected by senate 3x
- **1974** — The double dissolution election led to the passage of universal health care legislation
- **1975** — After the change of government, free health care was restricted to retirees
- **1984** — Medicare officially established, and all people enjoyed free public hospitals & subsidized medical care




| Hybrid System: **Beveridge + Bismarck Model** | |
|---|---|
| **Medicare:** Universal health insurance | **PHI:** Private Health Insurance |
| **Funding Source:** General tax + Medicare Levy (2% personal income tax) | **Funding Source:** Private insurance+ government subsidies |
| **Coverage:** Public,100% | **Coverage:** Private, supplementary, ~50% |

- The federal government sets policy and provides funding
- The state government operates hospitals and major medical services
- Local governments provide community health and preventive care
- Private Health Insurance Rebate
- Medicare Levy Surcharge (MLS)

# Overview of the WHO 6 Building Blocks and Comparison of Australia against the G7 Nations



1

# Service Delivery

*Australia's healthcare system, Medicare, is a universal public health insurance program funded by taxes and a government levy, providing free hospital care and extensive coverage for physician services, pharmaceuticals, and other services to citizens (3).*

# Australia's success in reducing sentinel events through accountability and strong safety standards

Key Indicator

## The number of the sentinel events in Australia's health system

**Sentinel events:** a serious, preventable incident in healthcare that causes significant harm or death to a patient

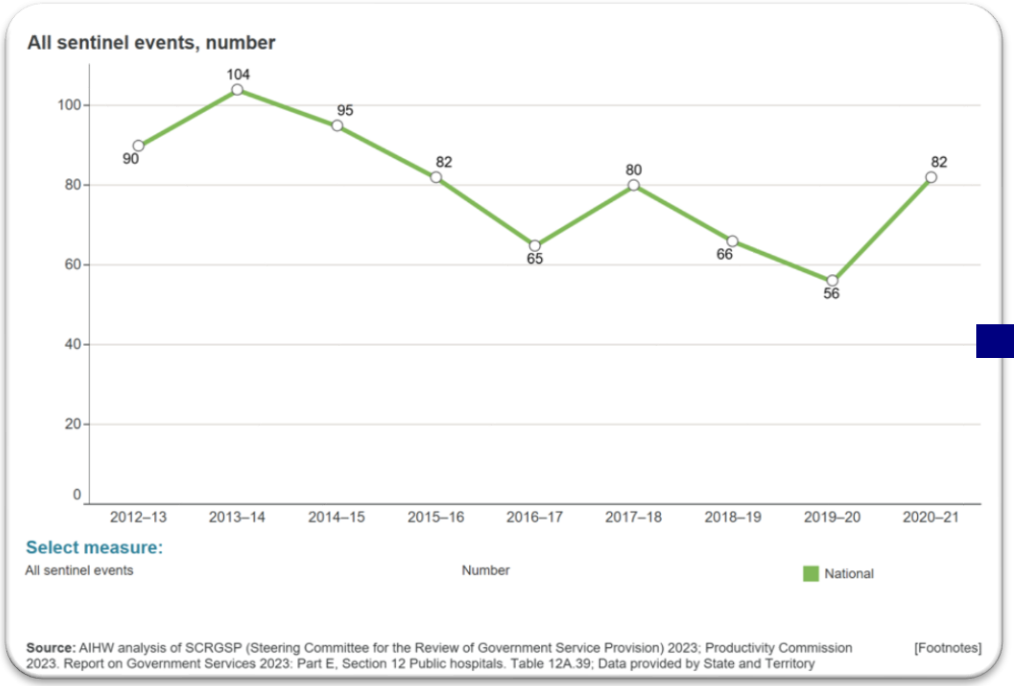



***Figure 1:*** *The number of sentinel events from 2012 to 2021 in Australia*

### Strength 1: **Improved Patient Safety**

The decline in sentinel events reflects the success of Australia's strong safety and quality standards, which contribute to a safer healthcare environment and reduce preventable harm (4,5)

### Strength 2: **Accountability and Continuous Improvement**

The Australian Commission on Safety and Quality in Health Care's rigorous enforcement of national safety protocols has fostered a culture of accountability, ensuring that healthcare providers consistently adhere to high standards (5)

# Rising life expectancy in Australia: A sign of effective health system and quality

Key Indicator

## Life expectancy at birth

**Life Expectancy:** The average number of years a person could expect to live if they experienced the age/sex specific death rates that applied at their birth throughout their lifetime.

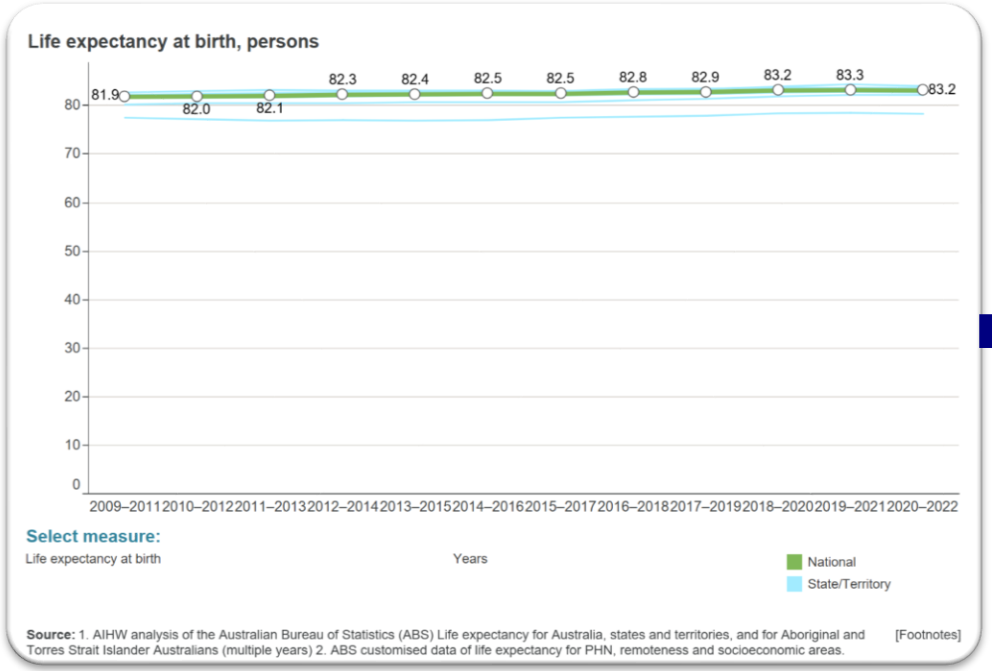



*Figure 2: The average number of life expectancy at birth from 2009 to 2022 in Australia*

### Strength 3: **Stable and Rising Life Expectancy: A Sign of Healthcare Progress**

Stable yet increasing life expectancy in Australia indicates the effectiveness of public health policies, healthcare access, and disease prevention efforts, reflecting overall improvements in healthcare and living conditions (5)

# Limitations Translate into Bottlenecks

**Limitation 1** Health System Inefficiency



**Limitation 2** Accessibility of Screening



**Limitation 3** Under-Resourcing of Acute Care Bed Capacity



Key Indicator

## Waiting times for elective surgery: Waiting times in days

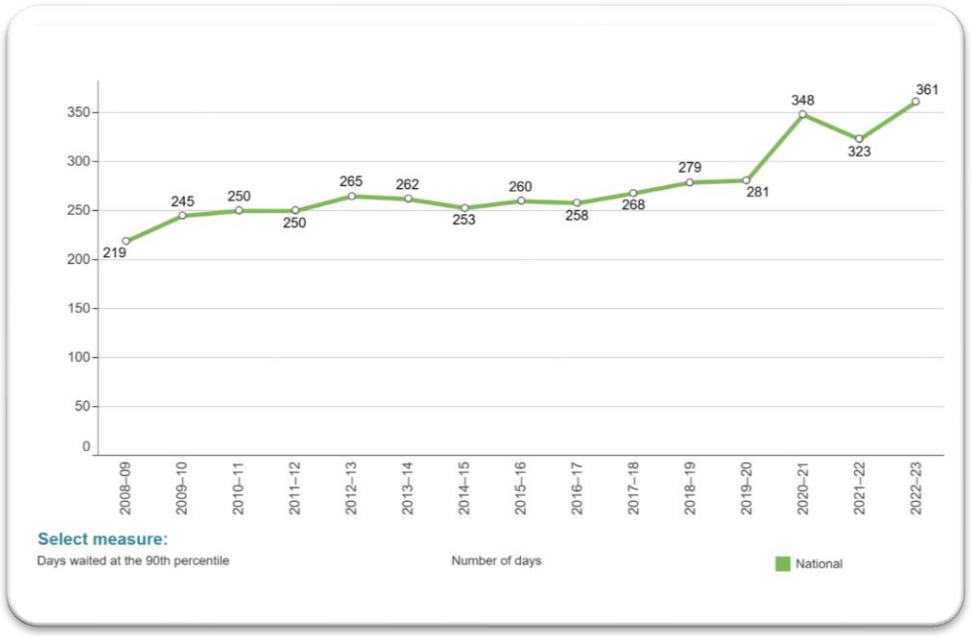



*Figure 3: waiting times for elective surgery: waiting time in days in Australia*

### Bottleneck 1: **Long Waiting Hours for Elective Surgery**

The waiting time at the 90th percentile has shown a general increase from **219 days** in 2008–09 to **361 days** in 2022–23.

The waiting time indicate a backlog in elective surgeries, leading to delayed treatments and potential worsening of patients' conditions.

The rising waiting times highlight systemic healthcare inefficiencies, indicating a need for better hospital capacity planning, workforce investment, and service delivery optimization to ensure timely and equitable access to elective surgical care.

# Accessibility of screening

Key Indicator

## Cancer screening participation, age-standardised rate (%)

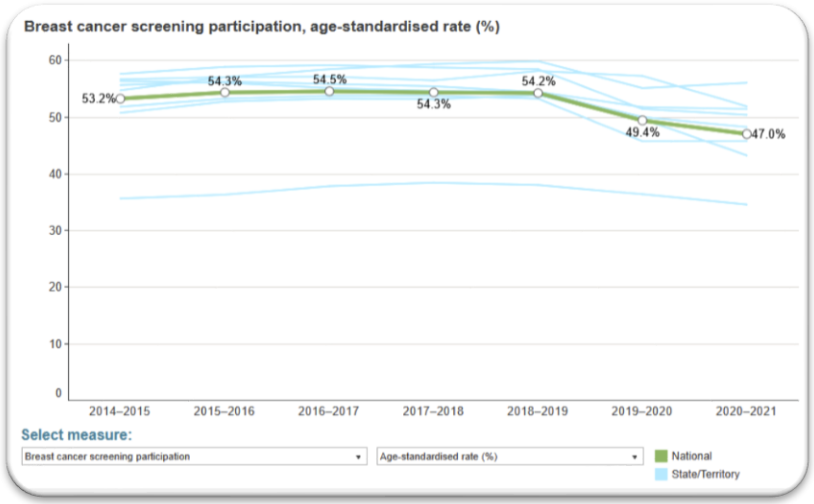



***Figure 4:*** *Breast cancer screening participation, crude rate (%)*

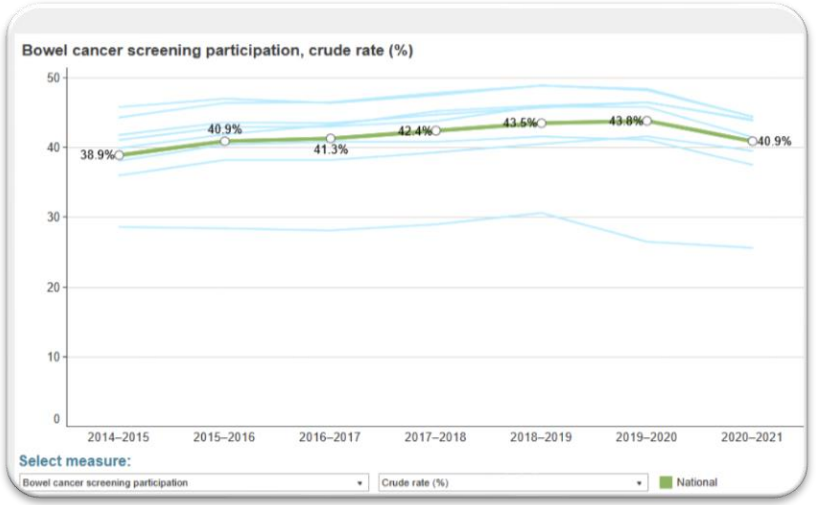



***Figure 5:*** *Bowel cancer screening participation, crude rate (%)*

### Bottleneck 2: **Lower Participation in Cancer Screening**

Accessibility: Structural barriers such as difficulty navigating the health system, lack of transportation, and long distances to screening services contribute to lower participation rates, particularly in socioeconomically disadvantaged and rural communities

# Under-resourcing of acute care bed capacity

Key Indicator

## Percent of emergency department patients waiting for admission suffering access block

**NOTE:** Access block refers to the situation where patients who have been admitted and need a hospital bed are delayed from leaving the emergency department (ED) for more than eight hours

### Bottleneck 3: **Access Block and ED Overcrowding**

**Key Drivers**

- **Rising Demand**: ED presentations in Australia surged by 14% (2011–2019), outpacing population growth (1.6% annually). Concurrently, admissions requiring beds rose by 25%, straining a shrinking bed base.
- **Declining Resources**: Hospital beds per 1,000 people fell from 2.65 (2011–12) to 2.53 (2018–19), reverting to 1990s levels despite higher acuity and demand.

**Consequences**

- **Patient Harm**: Mortality risk jumps by 67% for patients waiting >12 hours for admission. ED overcrowding (≥10% bed-blocked patients) raises mortality by 10% for new arrivals.
- **ED Overload**: Access-blocked patients consume ~33% of ED workloads, diverting resources from urgent care and worsening delays.

2

# Health Workforce

*The health workforce in Australia is large and diverse and includes a wide range of professionals and support staff working to provide healthcare services to the population. These include health practitioners registered with the Australian Health Practitioner Regulation Agency (Ahpra) as well as other health professionals and health support workers.*

# Australia's Healthcare Workforce: Accessibility, Diversity, and Sustainability

Key Indicator

## The number of full-time equivalent (FTE) health professionals per 100,000 population

**NOTE: FTE is based on 40 hours per week for medical practitioners and 38 hours per week for all other professions.**

| Profession | Measure | 2013 | 2022 | % change |
|---|---|---|---|---|
| Allied health[a] | Number of practitioners | 108,680 | 180,924 | 66.5 |
| | FTE total | 98,545 | 169,526 | 72.0 |
| | FTE per 100,000 population | 426 | 652 | 53.1 |
| Dental Practitioners [b] | Number of practitioners | 17,874 | 22,964 | 28.5 |
| | FTE total | 16,604 | 21,157 | 27.4 |
| | FTE per 100,000 population | 72 | 81 | 12.5 |
| Medical Practitioners (excluding GP) | Number of practitioners | 56,173 | 79,273 | 41.1 |
| | FTE total | 62,676 | 85,189 | 35.9 |
| | FTE per 100,000 population | 271 | 328 | 21.0 |
| General Practitioner (GP) | Number of practitioners | 26,235 | 32,635 | 24.4 |
| | FTE total | 25,706 | 29,626 | 15.2 |
| | FTE per 100,000 population | 111 | 114 | 2.7 |
| Nurses and Midwives | Number of practitioners | 295,060 | 372,759 | 26.3 |
| | FTE total | 267,164 | 339,883 | 27.2 |
| | FTE per 100,000 population | 1,155 | 1,307 | 13.2 |
| All professions | Number of practitioners | 503,995 | 688,555 | 36.6 |
| | FTE total | 470,695 | 645,381 | 37.1 |
| | FTE per 100,000 population | 2,035 | 2,482 | 22.0 |

***Table 1:*** *Key workforce statistics by health profession FTE, 2013 & 2022*

### Strength 1: **General Accessibility**

Increased FTE health professionals per 100,000 population (**+22%**) has enhanced healthcare accessibility, improving doctor-to-patient and patient-to-nurse ratios, reducing wait times, and strengthening emergency response.

### Strength 2: **Improved Diversity**

Growth in Allied Health and Dental Practitioners has expanded mental health, physiotherapy, and rehabilitation services, improving integration of primary care and specialized treatment for comprehensive patient care.

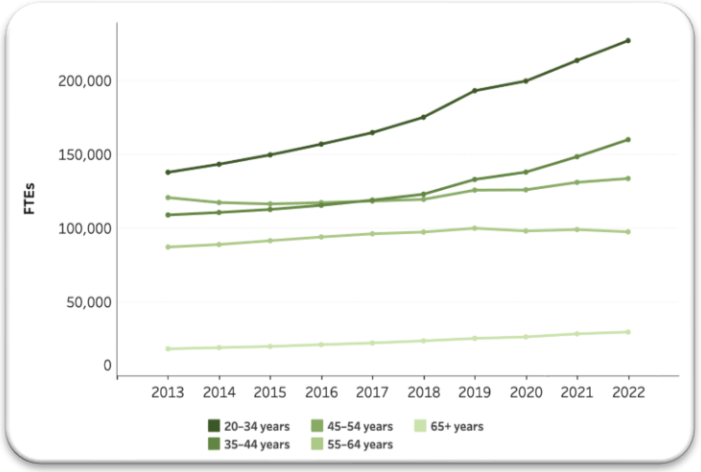



***Figure 1:*** *Total FTE and FTE rates, by age, 2013 to 2022*

### Strength 3: **Growing Dynamism**

A **47%** FTE increase for ages 20–34 and **31%** for ages 35–44 ensures long-term healthcare sustainability, with younger professionals replacing retirees and maintaining service availability.

# Limitations Translate into Bottlenecks

**Limitation 1** Regional Disparities



**Limitation 2** Unattractive Working Conditions



**Limitation 3** Overbearing Specialist Competition



Key Indicator

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**NOTE: FTE is based on 40 hours per week for medical practitioners and 38 hours per week for all other professions.**

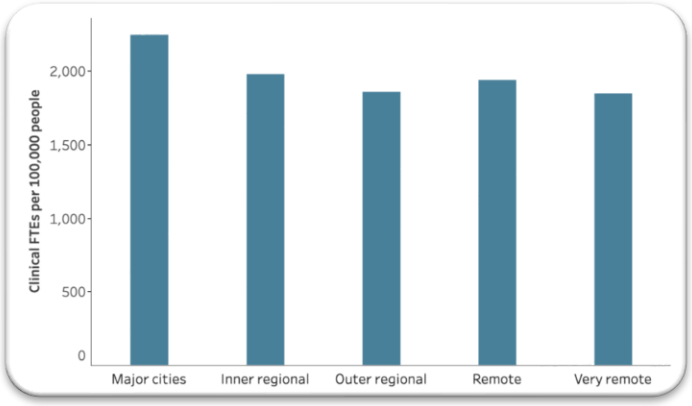



***Figure 2:*** *Total clinical FTEs and clinical FTE rate, by profession and remoteness, 2013 to 2022*

### Bottleneck 1: **Unequal Distribution**

Lower clinical FTE rates in rural areas limit access to essential care, worsening health outcomes. **Overconcentration in major cities** deepens disparities between urban and remote communities.

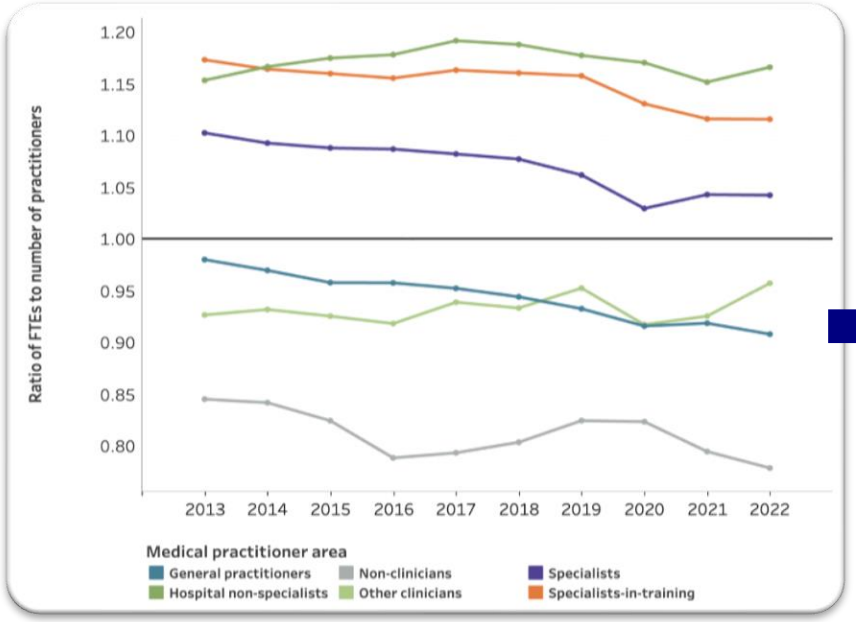



***Figure 3:*** *Ratio of FTE total to number of practitioners among medical practitioners, by job area, 2013 to 2022*

### Bottleneck 2: **Long Working Hours**

Between 2013 and 2022, **medical practitioners (excluding GPs)** were the only health professionals with total FTE exceeding actual numbers, averaging over 40 hours per week.

Specialists, specialists-in-training, and hospital non-specialists had **the highest workloads**, while allied health, nursing, and dental practitioners saw more part-time arrangements.

Key Indicator

## Proportion of vacancies filled & number of suitable applicants per vacancy for Health Professionals

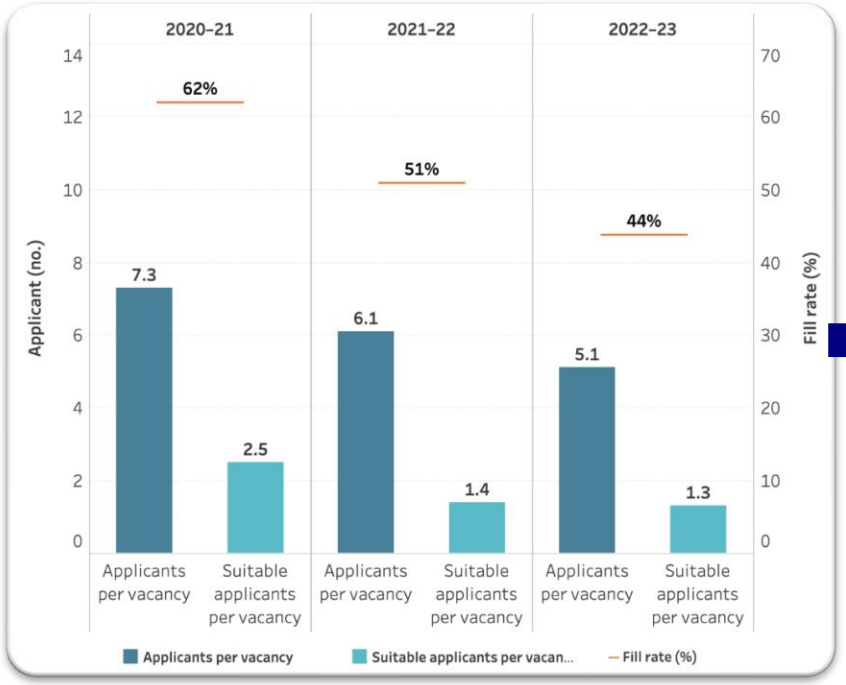



*Figure 6: Proportion of vacancies filled and number of suitable applicants per vacancy for Health Professionals, 2020–21, 2021–22 & 2022–23*

### Bottleneck 3: **Shortages in health workforce supply**

In 2023, **82%** of health professional occupations faced shortages, impacting general practice, mental health, nursing, and allied health.

The fill rate dropped to **44%** (from 60% in 2022–23), with only 1.3 suitable applicants per vacancy.

A key issue is the lack of specific skills and experience, posing risks to **workforce sustainability**.

# Overbearing Specialist Competition

Key Indicator

## Population percentage of internationally trained Medical Practitioners

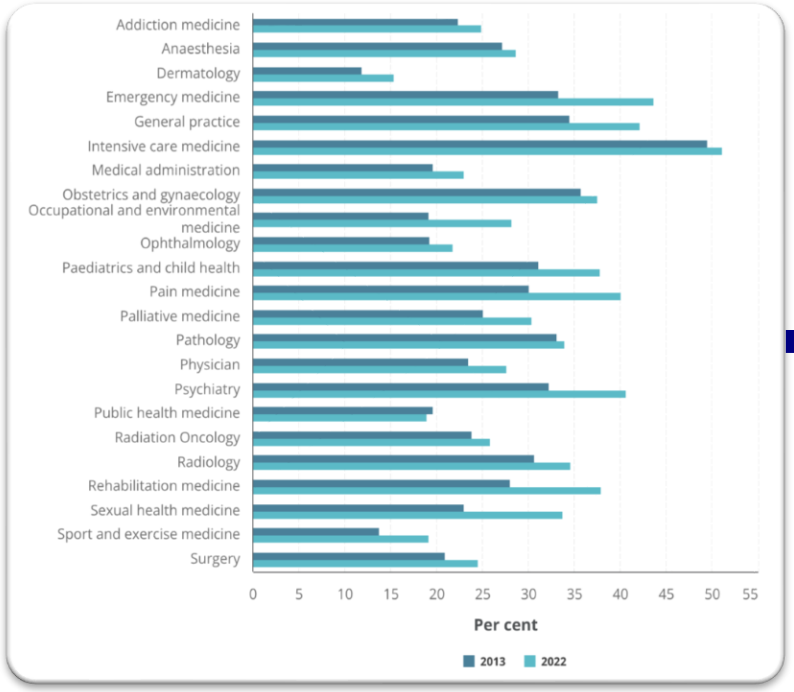



*Figure 4: Overseas trained health workforce, by specialities, 2013 and 2022*

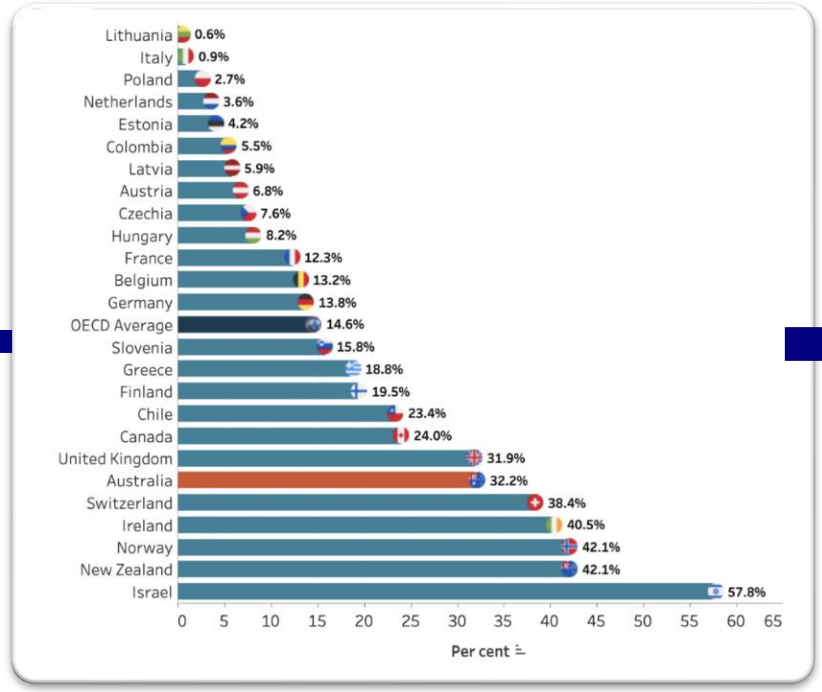



*Figure 5: Proportion of overseas trained doctors, by OECD country, 2021*

*NOTE: OECD refers to the Organisation for Economic Co-operation and Development countries, a group of 38 member nations committed to democracy and a market economy.*

### Bottleneck 3: **Heavy Reliance on Overseas Health Workforce**

Australia's reliance on internationally trained doctors **rose to 32% (2018–2022) from 29% in 2013**, with Intensive Care (51%), Emergency Medicine (44%), and General Practice (42%) most affected.

Ranking **6th highest in the OECD** for overseas-trained doctors and 21st lowest for domestic training, this dependency highlights workforce vulnerability.

# HSPA Overview of Australia's Health Care System

## Service Delivery

| Key Indicator | Characteristics | Evaluation |
|---|---|---|
| The Number of the Sentinel Events in Australia's Health System | Improved Patient Safety | Strengths |
| | Accountability and Continuous Improvement | |
| Life Expectancy at Birth | Stable and Rising Life Expectancy: A Sign of Healthcare Progress | |
| Waiting times for elective surgery: waiting times in days | Long Waiting Hours for Elective Surgery | Bottlenecks |
| Cancer screening participation, age-standardised rate (%) | Lower Participation In Patient Screening | |
| Percent of Emergency Department patients waiting for admission suffering access block | Access Block and ED Overcrowding | |

## Health Workforce

| Key Indicator | Characteristics | Evaluation |
|---|---|---|
| The Number of Full-Time Equivalent (FTE) Health Professionals per 100,000 Population | General Accessibility | Strengths |
| | Improved Diversity | |
| | Growing Dynamism | |
| | Regional Disparities | Bottlenecks |
| | Unattractive Working Conditions | |
| Proportion of Vacancies Filled & Number of Suitable Applicants per Vacancy for Health Professionals | Shortages in health workforce supply | |
| Population Percentage of Internationally Trained Medical Practitioners | Heavy Reliance on Overseas Health Workforce | |

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# Thank You

**Any Questions?**

Health Systems Performance Assessments (HSPA) Team
Emmanuel, Siyuan, Taras, Ria, Jiawei, & Shelly

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6. Australian Institute of Health and Welfare. Health workforce [Internet]. Canberra (AU): AIHW; 2024 Jul 2 [Accessed 2025 Feb 16]. Available from: https://www.aihw.gov.au/reports/workforce/health-workforce
7. Azar, D. et al. (2021) “Barriers and facilitators to participation in breast, bowel and cervical cancer screening in rural Victoria: A qualitative study,” Health Promotion Journal of Australia, 33(1), p. 272. Available at: https://doi.org/10.1002/HPJA.478.
8. Jones, P.G. and van der Werf, B. (2021) “Emergency department crowding and mortality for patients presenting to emergency departments in New Zealand,” EMA - Emergency Medicine Australasia, 33(4), pp. 655–664. Available at: https://doi.org/10.1111/1742-6723.13699.
9. May 2023 Policy on Standard Terminology - Australasian College for Emergency Medicine
10. Position Statement Access Block - Australasian College for Emergency Medicine

# Appendix A Comparing Australia's Health System to the Group of 7

| Building Block | Indicator | G7 Range | Australia |
|---|---|---|---|
| **Service Delivery** | UHC Service Coverage Index (3.8.1) | 83 - 91(JPN - CAN) | 87 |
| | Hospital beds per 1000 population | 2.4 - 12.7 (GBR - JPN) | 3.8 |
| | Facilities offering maternal care (%) | High | High - 94% |
| **Health Workforce** | Physicians per 1,000 population | 2.6 - 4.5 (JPN - GER) | 4.0 |
| | Nurses and Midwifes per 1,000 population | 6.8 - 13.4 (ITA - USA) | 13.5 |
| | Dentists per 1,000 population | 0.49 - 0.88 (GBR - ITA) | 0.64 |
| | Pharmacists per 1,000 population | 0.67 - 2 (GER - JPN) | 0.94 |
| **Health Information** | Health information system performance index | Varied | Advanced (EHR adoption > 90%) |
| **Essential Medicines** | Availability of essential medicines in public facilities | ~70 - 95% (USA - FRA) | 85 - 90% |
| **Health Financing** | Total health expenditure (% GDP) | 8.9 - 17.6 (ITA - USA) | 10.6 |
| | Domestic general government health expenditure as percentage of general government expenditure (%) | 11.9 - 24.7(ITA - USA) | 20.2 |
| | Out-of-pocket spending (% total health expenditure) | 8.9 - 22.7 (FRA - ITA) | 15.3 |
| **Leadership/Governance** | Policy index (UHC, National Health Strategy) | Strong (GBR - Centralised, GER - Federal) | Strong (Decentralised) |